



جوستاف روسي انترناشيونال - مصر (EPE) * طلب علاج أول مرة / متردد

إسم المريض ربيعاً : راوية عبدالنواب مصطفى -

العنوان : نعيم

الحي / المدينة : اهناسيا

المحافظة : بني سويف

تاريخ تسجيل الإستمارة : 01-10-2026

رقم البطاقة : 25401032201104 رقم التليفون : 01112300750

العمر (تاريخ الميلاد) : 03-01-1954 المهنة : بدون عمل

الإقرار

أقر أنا الموقع أدناه أنني لا اتمتع بأي نظام علاج مثل :- التأمين الصحي-القوات المسلحة-الشرطة-الهيئات-المؤسسات أو أي نظام يتعارض مع نظام العلاج علي نفقة الدولة- وسوف اتحمل كافة نفقات العلاج في حالة ثبوت غير ذلك

الإحصائي الاجتماعي (.....) التوقيع: (.....)

المقرر بما فيه (.....) التوقيع: (.....)

التشخيص الإكلينيكي : سرطان بالثدي (مبادرة)

نتائج الأبحاث : (مرفق)

التشخيص النهائي : سرطان بالثدي (مبادرة)

خطة العلاج : أبحاث أورام مزيد من الأبحاث و الاشعات [7954] قسم العلاج : الاورام

التعليق على التشخيص :

تقرير اللجنة الثلاثية المتخصصة

الاسم : د/محمد كيلاني الوظيفة : التوقيع :
 الاسم : د/ندى حسن الوظيفة : التوقيع :
 الاسم : د/نرمين رمضان الوظيفة : التوقيع :

أطلعت اللجنة علي لائحة العلاج الخاصة بالمجالس وتلتزم جهة العلاج بلائحة المجالس الطبية. كما تقرر اللجنة بأن البيانات المدونة في هذا النموذج صحيحة.

تعتمد صحة البيانات المرفقة رئيس اللجنة ومدير المستشفى.

تعهد المستشفى أن لا يتم تنفيذ القرار إلا بعد مراجعة الملف الطبي للمريض وإعادة مناظرة المريض بالمستشفى قبل الصرف.

أعضاء اللجنة

..... /د. /..... /د. /..... /د. /..... /د.

قرار اللجنة

التشخيص النهائي :
 خطة العلاج :
 المستشفى : قسم :
 جهة العلاج : نفقات :
 ملاحظات :

ختم
شعار الجمهورية

مدير عام
مديرية الشؤون الصحية

رئيس
المجلس الطبي المتخصص

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Name: - راويه عبدالتراب مصطفى National ID: 25401032201104	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
--

تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
--

المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Further investigations and imaging

التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين



مرمى اورام دارى اسلام
Patient name: راية عبد التواب مصطفى

Date: 2/3/2022

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular parenchyma density (ACR B).
- Right breast LOQ middle third irregular spiculated high density mass, measuring about 64 x43 x50mm (in CC&MLO views).
- Left UOQ IMLN.
- No left suspicious mass or microcalcification
- **Complementary ultrasound** showed right breast LOQ at 8 o'clock irregular spiculated hypoechoic mass, with internal vascularity on color Doppler application, about 5.4mm from the overlying skin, 15mm from the nipple, measured about 35 x18mm, consistent to the palpable lump.
- No left breast suspicious cystic or solid mass lesions.
- Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Left Nonspecific axillary LN.
- Right multiple Pathological axillary lymph nodes, level I lesion than 3 LNs. The largest measuring about 30 x16mm with irregular in shape and effaced hilum.
- Free supra and infra clavicular lymph nodes.

Opinion:

- Right breast 8 o'clock highly suspicious mass with ipsilateral pathological axillary lymph nodes (level I) for histopathological correlation. (BIRADS 5),
- left breast benign findings (BIRADS 2)

Best regards.

Dr. Nehal Hamdey, FEBR, MSc

21 / 3 / 20

Clinical Data

Recommendations: Left Breast Mass 2x1.5 cm, (3-5 cm) (3/15)
 Pt2 had BCS + ALND 7/3/22 For post operative Management
 → send PT2 dx (4/15)

11-4-212

MDT OUTCOME & PLAN

- For CT CAP & contrast
- Bone Scan
- Echo & labs $\rightarrow$ EF: 38%
- Bring IHC result

- 4-21-22

for adj chemo Taxoten + Endoxan x 4 G doc then
PORT

SIGNATURE			
MEDICAL ONCOLOGY	SURGERY	RADIOLOGY	PATHOLOGY
			

HEALTH INSURANCE ORGANIZATION CENTER DEPARTMENT OF INFORMATION AND DECHON SUPPORT		الهيئة العامة للتأمين الصحي الادارة المركزية لتكنولوجيا المعلومات ودعم القرار
جبهة الاصدار : رئاسة الهيئة	تاريخ الاصدار : 2023-1-31	
الى من يهمه الامر		
الهيئة العامة للتأمين الصحي تعمل من خلال قوانين منظمة للانتفاع الخاضعين لقانون التأمين الاجتماعي رقم 79 لسنة 1975 المعدل بالقانون 25 لسنة 1977 "القوى العاملة والمعاشات والارامل" وكذلك قانون 99 لسنة 1992 الخاص بطلاب المدارس و قانون رقم 86 لسنة 2012 " الاطفال دون السن المدرسي" وقانون رقم 23 لسنة 2012 الخاص بالمرأة المعيلة . وطبقا لقاعدة البيانات المرسلية من التأمينات الاجتماعية وطبقا لقاعدة البيانات المرسلية من الضمان الاجتماعي والبحث تبين أن		
السيد (ة) راوية عبدالنواب مصطفى بطاقة رقم 25401032201104 وطبقا الى قانون 112 عمالة غير منتظمة يعتبر من الفئات غير المتفله بالتأمين الصحي وشكرا		
الموظف المختص  محمد منصور	تاريخ الطباعة : 2023-1-31	
افادة مركز نظم المعلومات يعتمد،،،		
		مسلسل 390408
الخطيب ساري لمدة ثلاثة أشهر من تاريخ الإصدار إلا في حالة تغير حاله المواطن من قبل الهيئة القومية للتأمينات الاجتماعية أو وزارة التضامن الاجتماعي أو وزارة التربية والتعليم		
الادارة المركزية لتكنولوجيا المعلومات ودعم القرار		



Patient Name	: الأستاذة / رايه عبدالنواب مصطفى	Request No.	: 13002289
Patient No.	: 25401032201104	Request Date	: 21/03/2022
Path No.	: SR-211/22	Verification Date	: 29/03/2022
Sex	: Female	Financial Category	: اورام دار السلام - نفقة الدولة
Age	: 68 Y	Department	: Out Patient
Referreing Doc :		Registered by	: Ali mostafa

Pathology Report

Clinical Data:

Rt breast cancer.

Nature of the specimen: Slides revision

Gross Examination: Eight slides and eight blocks labelled by (4103) and one block labelled by (3003LN) were received.

Microscopic examination:

- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism + mitosis): G III (3+3+3).
- Ductal carcinoma in situ: Focal in less than 5%.
- Lymph node involvement: (4/15), with extracapsular invasion in one LN.

Diagnosis:

Rt breast mass, Rt axillary LN, FNAC and wide local excision ,slide revision

- Invasive duct carcinoma, Grade III.
- LN involvement : (4/15) , with extracapsular invasion in one LN.
- For IHC : ER, PR, HER-2, Ki 67.

Prof.Dr. Eman Gouda

Verified by :

dr.esraa magdy

Printing Date : 29/03/2022 13:50:45

Page 1 of 1

Printing By : Ali mostafa



Patient Name: Rawia Abd Al Tawwab Moustafa	Date : 17/09/2024
Exam : C.T CHEST WITH CONTRAST	Age : 71 Year(s)
Medical ID: 1003/2022	Pat. ID: 28105

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -

1. Chest: As compared to the prior study dated 19/5/2024

- **Stationary course of size and numbers of bilateral** tiny pulmonary nodules, largest one at anterior segment of right lower lobe still measures 8 mm.
- **Progression size of** previous bilateral hilar and mediastinal lymph nodes are noted. the largest is seen at right para tracheal group measures 27x24 mm as compared to 18 x 26 mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver still enlarged in size with stable of left hepatic lobe segment IV hypodense small hypo-enhancing focal lesion measures 8 x 9 mm.
- Patent homogenously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.

Dr. Nehal Hamdy
Patient name: رايوة عبد القواب مصطفى

Date: 2/3/2022

Combined bilateral breast mammography and complementary ultrasound

Indication:

Right breast palpable lump.

Technique

- Bilateral MLO and CC views
- Bilateral 3D MLO DBT (Digital Breast tomosynthesis)
- Bilateral breast ultrasound

Report:

- Scattered fibro glandular parenchyma density (ACR B).
- Right breast LOQ middle third irregular spiculated high density mass, measuring about 64 x 43 x 50mm (in CC&MLO views).
- Left LOQ IMLN.
- No left suspicious mass or microcalcification
- Complementary ultrasound showed right breast LOQ at 8 o'clock irregular spiculated hypoechoic mass, with internal vascularity on color Doppler application, about 5.4mm from the overlying skin, 15mm from the nipple, measured about 35 x 18mm, consistent to the palpable lump.
- No left breast suspicious cystic or solid mass lesions.
- Normal skin thickness, and nipple-areola complex with average caliber of the retro-areolar central ducts.
- Left Nonspecific axillary LN.
- Right multiple Pathological axillary lymph nodes, level I lesion than 3 LNs. The largest measuring about 30 x 16mm with irregular in shape and effaced hilum.
- Free supra and infra clavicular lymph nodes.

Opinion:

- Right breast 8 o'clock highly suspicious mass with ipsilateral pathological axillary lymph nodes (level I) for histopathological correlation. (BI-RADS 5),
- left breast benign findings (BI-RADS 2)

Best regards,
Dr. Nehal Hamdy, FEBR, MSc

Patient Name :	الاستاذة / رويدا عبدالقواب مصطفى	Request No. :	13002289
Patient No. :	25401032201104	Request Date :	21/03/2022
Path No. :	SR-211/22	Verification Date :	10/04/2022
Sex :	Female	Financial Category :	اورام دار السلام - نفقة الدولة
Age :	88 Y	Department :	Out Patient
Referring Doc. :		IHC No. :	
		Registered by :	All mostafa

IMMUNOHISTOCHEMISTRY REPORT

Clinical Data:

Rt. breast cancer.

Nature of the specimen: Slides revision.

Gross Examination: Eight slides and eight blocks labelled by (4103) and one block labelled by (3003LN) were received.

Microscopic examination:

- Histological type: Invasive duct carcinoma.
- Grade (differentiation+ nuclear pleomorphism + mitosis): G III (3+3+3).
- Ductal carcinoma in situ: Focal in less than 5%.
- Lymph node involvement: (4/15), with extracapsular invasion in one LN.

Diagnosis:

Rt breast mass, Rt axillary LN, FNAC and wide local excision, slide revision

- Invasive duct carcinoma, Grade III.
- LN involvement: (4/15) with extracapsular invasion in one LN.
- IPT (appropriate tissue control was reviewed):

Sections prepared from paraffin block for Immune- staining using Bench Mark ULTRA system for ER,

PR,HER2, ER clone (SP1), PR clone (1E2), HER-2 CLONE (4B5), & Ki67 (30-9)

- ER: Strong nuclear reaction in about 95% of tumour cells. Positive, score (8/8).
- PR: Weak nuclear reaction in about 10% of tumour cells. Positive, score (3/8).
- Her-2: No membrane reaction in tumour cells. Negative, score 0
- Ki67: 40% nuclear reaction

Prof. Dr. Eman Gouda

Dr. Abdurrahman Ibrahim

Verified by :

abdurrahman ibrahim

Printing Date : 11/04/2022 09:13:33

Page 1 of 2

Printed By : amrwa monamed youss



Patient Name: Rawia Abd Al Tawwab Moustafa	Date : 22/05/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 71 Year(s)
Medical ID: 1003/2022	Pat. ID: 28105

POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST, ABDOMEN & PELVIS REVEALED: -
As compared to the prior study dated 15/02/2025

Chest:

- **Regressive course of the size and number of the bilateral tiny pulmonary nodules**, largest one at anterior segment of right lower lobe still measures 6 mm compared to 9 mm.
- **Rather stationary** Regrading size of previous bilateral hilar and mediastinal lymph nodes are noted. the largest is seen at right para tracheal group measures 27x24 mm.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size and shape with no gross abnormality of the cardiac chambers. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.
- Bilateral elastofibroma dorsi.

Abdomen and pelvis:

- The liver still enlarged in size with currently noted hepatic dome small enhancing focal lesion measuring 8 mm.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Cholecystectomy clip is noted.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- No pathological abdominal or pelvic lymph nodes.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and right supra-renal.
- Stable size of left supra-renal mass lesion measures 3.5 x 2.8 cm.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.

Date : 31/01/2023	Patient Name: RĀWYA MOSIĀFA ABDELIWAB
Age :	Exam : Item 1
Pat. ID: QC_28105	Medical ID:

Name : rawia mostafa
Date: 31 January 2023

Plain X-ray Chest (PA view) revealed:

- Enlarged left hilar shadow? LN for CT evaluation..
- Clear both lung fields.
- Clear both costo-phrenic angles.
- Normal cardiac size & configuration.

Signature Dr. Doaa Abdelaziz
31/01/2023 2:54 PM
Signature Dr. Doaa Abdelaziz
31/01/2023 2:54 PM

A, RWEA, Patient ID: 123014521 DOB: 03-Jan-1954 Date: 24-Jul-2023

Patient Lastname: A
Patient Firstname: RWEA
HIS Patient ID: 123014521
RIS Patient ID: 332266
Date of Birth: 03/01/1954

Procedure: 70030, Bone Scan
Medical Question:
Clinical Comment:

===== REPORT TEXT =====

99mTc MDP Bone Scan Report (24-7-2023)

* Whole Body Anterior and Posterior Planar Views Acquired 2Hrs. Post I.V Injection of 20mCi of 99mTc MDP Revealed:

" No definite evidence of active osseous deposits could be seen over the surveyed skeleton, however warm osseous changes are noted overlying multi-level dorso-lumbar vertebrae (mainly over DV9 and most of the lumbar vertebrae)? Exaggerated degenerative changes? Otherwise, warranting further radiological correlation.

" Active arthritic changes are noted over both shoulder, both knee and both ankle joints.

" Both kidneys are visualized.

Conclusion:

" No definite evidence of active metastatic osseous deposits could be seen all over the surveyed skeleton, however radiological correlation for the thoracic active osseous changes may be advised to confirm/ exclude any developing serious pathology therein.

Much Obligated
Dina Thabet
Dr. Nada Fadl, MSc

===== REPORT TEXT ENDE =====



Patient Name: Rawia Abd Al Tawwab Moustafa	Date : 17/09/2024
Exam : C.T CHEST WITH CONTRAST	Age : 71 Year(s)
Medical ID: 1003/2022	Pat. ID: 28105

- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and right supra-renal. Stable size of left supra-renal mass lesion measures 3.5 x 2.8 cm.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic masses or collections.

3. The scanned skeleton:

- The scanned skeleton still shows no worrisome lytic or sclerotic lesions, and advanced dorso-lumbar spondylosis with anterior slippage of L5 over S1 vertebrae due to bilateral pars break.

OPINION:

Follow up status revealed:

- ✚ ***Mild progression regrading size of previous bilateral hilar and mediastinal lymph nodes.***
- ✚ ***Otherwise, stationary course rest of the study.***

Dr. Kholoud Khaled
Dr. Reham Anwar, FEBR, MD



Patient Name: Rawia Abd Al Tawwab Moustafa	Date : 22/05/2025
Exam : CT CAP+C (CT Chest Abd. Pelvis)	Age : 71 Year(s)
Medical ID: 1003/2022	Pat. ID: 28105

- No abdominal or pelvic masses or collections.

The scanned skeleton:

- The scanned skeleton still shows no worrisome lytic or sclerotic lesions, and still dorso-lumbar spondylosis with anterior slippage of L5 over S1 vertebrae due to bilateral pars break.

OPINION:

Follow up status revealed:

- Regressive size and number of the bilateral pulmonary nodules.***
- Currently noted hepatic dome enhancing focal lesion ?? flash filling hemangioma for close follow-up.***
- stationary course of the rest of the study.***

Dr. Mohamed Mohsen, M.D.

Dr. Mohamed Mohsen, M.D.